



PROCEDURE

Safety Net Meeting Process – Early Childhood Injury Proforma

Scope (Staff):	Consultant, Registrar, Social Work, Child Protection Unit (CPU) Team Leader
Scope (Area):	Perth Children's Hospital (PCH), Emergency Department (ED) and CPU

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

Disclaimer

These guidelines have been produced to guide clinical decision making for the medical, nursing and allied health staff of Perth Children's Hospital. They are not strict protocols, and **they do not replace the judgment of a senior clinician**. Clinical common-sense should be applied at all times. These clinical guidelines should never be relied on as a substitute for proper assessment with respect to the particular circumstances of each case and the needs of each patient. Clinicians should also consider the local skill level available and their local area policies before following any guideline. Read the full disclaimer here:

[CAHS | Child and Adolescent Health Service - CAHS Clinical Disclaimer](#)

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Aim

To reduce the chance of inflicted injury being overlooked among children (under two years) who present to the ED with physical injuries (including but not limited to bruises and burns), ingestions, or immersions.

To standardise the use of the [Early Childhood Injury Proforma \(MR301.3\)](#) and associated Safety Net meeting process within PCH.

Risk

Inconsistent use of the Early Childhood Injury Proforma may result in signs of child abuse going undetected. The Safety Net meeting is an additional process to safeguard children at risk

Background

In 2021-2022, the Coronial Review Committee (CRC) chaired by the Chief Medical Officer made recommendations to improve existing processes following the coronial inquiries of two infant deaths, where inflicted injury in non-ambulant children went unrecognised by health professionals, leading to delays in preventing further harm¹.

This procedure formalises and improves an approach that has been in place at the PCH Emergency Department (and previously PMH) for several years.

It is widely recognised that the use of standardised assessments in hospital Emergency Departments improves the detection of inflicted injury in infants and young children^{1,2} and thus activates a response for further assessment and safeguarding.

At PCH, the use of the *Early Childhood Injury Proforma (MR301.3)* is enhanced by additional Safety Net meeting procedures aimed to mitigate the risk of inflicted injury of a child (under 2 years) being overlooked and to provide timely intervention in instances where an injury proforma has not been completed, and/or where there are other child protection concerns identified.

The injury proforma that is an integral component of this procedure has been updated by the Child Protection Unit (CPU) based at PCH with modifications that are informed by evidence and best practice^{1,2} and align with the recommendations of the coroner.

An ethics approved research trial to validate the updated Early Childhood Injury Proforma was conducted at PCH to determine its reliability for both suspicion of inflicted injuries and other child protection concerns and confirmed the use of the Early Childhood Injury Proforma as best practice at PCH and across other WA Health Service Providers (HSPs).

Definitions

Early Childhood Injury Proforma: Approved PCH form to be used for any child under two years in age who presents to the PCH Emergency Department with any physical injury / ingestion / immersion (replaces the generically titled Injury Proforma).

Key points

- All children under the age of two years who present to the ED with an injury, ingestion, or immersion should have an [Early Childhood Injury Proforma \(MR301.03\)](#) completed by the attending Doctor at the time of their ED presentation
- Consider cultural needs of the family when undertaking the injury proforma assessment and provide additional family support if indicated.
- Any concerns raised through the completion of the Early Childhood Injury Proforma must be discussed with the most senior ED doctor in the department to decide on the appropriate action.
- Safety Net meetings provide additional scrutiny for children under the age of two years who have presented to ED in the previous week with an injury, ingestion or an immersion, to minimise the risk of overlooking possible inflicted injury and/or other child protection concerns.
- If for any reason an Early Childhood Injury Proforma is not completed at the time of the ED presentation, further evaluation is undertaken at the next Safety Net meeting to determine if there are any relevant risk factors or concerning features for further action.

Process

- The Safety Net meeting will consist of the following clinical representation (role specific):
 - Child Protection Unit Registrar and/or Consultant
 - Child Protection Unit Team Leader or Senior Social Worker
 - Emergency Department Registrar
 - Emergency Department Social Worker (optional)

Safety Net meetings will be held weekly and are co-chaired by ED / CPU Registrars. All meeting documentation is recorded directly into the ED Safety Net database.

Steps: Safety Net Meeting Procedure	Additional Information
1. Prior to Safety Net Meeting	
1. ED Trauma Registry Officer compiles a list of relevant medical records for review (i.e., all injury, ingestion or immersion presentations at ED in the past week for children < 2 years). This list is saved on the ED Safety Net Database. 2. ED Registrar reviews the medical records to determine if the Early Childhood Injury Proforma (Injury Proforma) has been completed. 3. Completed Injury Proformas ticked as 'not suspicious' and signed off by an ED Consultant or Senior Fellow are deemed 'Not for Action' (NFA). a) Medical records for patients who: 4. Do not have a completed Injury Proforma ○ Injury Proforma ticked as 'suspicious' or ○ Injury Proforma ticked 'not suspicious but child protection concerns' will be discussed at the Safety Net meeting. b) With the commencement of PCH Digital Medical Records (DMR) the Injury Proforma will be scanned into the DMR in the 24-48 hours from ED discharge*.	To note: EDIS may erroneously flag some cases as an injury on triage and the medical records requested as relevant for Safety Net review. These cases do not require an injury proforma and are removed from the ED Safety Net database. Examples include but are not limited to, conjunctivitis, septic arthritis, osteomyelitis, tumour. Refer to Appendix 1: Workflow CAHS Safety Net Meeting - Planning *The list of relevant medical records is compiled from the presentations to ED the preceding week and accommodate the time required for the Injury Proforma to be scanned into the DMR.

2. During Safety Net meeting	
5. Medical records are reviewed to ascertain if the injury is suspicious and if further information / investigation is required. 6. CPU will check if any patient has had previous contact with CPU and share this information as well as the outcome of any cases that have been referred to CPU prior to the meeting. a) All relevant case details including the outcome of the Safety Net review are entered onto the ED Safety Net database and recorded in the DMR for that ED presentation. b) CPU will provide further action for the “treat as suspicious” cases or if other child protection concerns have been raised.	Additional information can be sourced from EDIS, iCM, IMPAX/InteleConnect, BOSSnet etc. to determine if there are any other red flags of concern. If required, previous medical records (paper file) should be requested. Refer to Appendix 2: Workflow During CAHS Safety Net Meeting
3. Post Safety Net meeting	
a) ED Registrar records the cases with NFA outcome and those recorded ‘to be followed up by CPU’ as ‘resolved’ onto the ED Safety Net database. 7. For cases that require CPU follow up, the CPU Team Leader allocates the action to the CPU Social Worker / Doctor on duty, and this is recorded on the CPU Duty Log document. 8. If the family are contacted and/or a clinic is organised, CPU will enter this onto WebPAS. b) Outcome from further CPU assessment will be shared the following meeting.	To encourage and support future compliance, the ED Registrar will send a generic reminder email to any ED doctor who did not complete the injury proforma in the previous week for a child under 2 years old with injury / immersion/ ingestion presentation. Refer to Appendix 3: Workflow CAHS Safety Net Meeting – Post meeting

Record Keeping

- The Emergency Department captures audit data on a shared spreadsheet ('ED Safety Net database') accessible to all Safety Net meeting members.
- Outcomes that are discussed are recorded on the spreadsheet and the completion of the Safety Net process is also recorded in the DMR. The CPU is responsible for coordinating additional follow-up as needed.
- Refer to Appendices 1 to 3 for data entry and record keeping requirements as they occur throughout the Safety Net process.
- Compliance in completion rates of the injury proforma can be audited to inform and support consistent adherence of the form in ED.

References and related external legislation, policies, and guidelines
9. From Death We Learn 2020 (2021 Edition). Coronial Liaison Unit. Progress Report for Health-Related Coronial Recommendations - August 2022. Perth, Western Australia; 2022. From Death We Learn 2020 (2021 Edition) (health.wa.gov.au)
10. Louwers EC, Korfage IJ, Affourtit MJ, Scheewe DJ, van de Merwe MH, Vooijs-Moulaert FA, et al. Detection of child abuse in emergency departments: a multi-centre study. Arch Dis Child. 2011;96(5):422-5.
11. Louwers EC, Korfage IJ, Affourtit MJ, Scheewe DJ, van de Merwe MH, Vooijs-Moulaert AF, et al. Effects of systematic screening and detection of child abuse in emergency departments. Pediatrics. 2012;130(3):457-64.
12. Sheets LK, Leach ME, Koszewski IJ, Lessmeier AM, Nugent M, Simpson P. Sentinel injuries in infants evaluated for child physical abuse. Pediatrics . 2013 Apr;131(4):701 7.
13. Mary Clyde Pierce, Kim Kaczor, Douglas Lorenz et al. Validation of a Clinical Decision Rule to Predict Abuse in Young Children Based on Bruising Characteristics. JAMA 2021 Apr

Related CAHS internal policies, procedures and guidelines
Burns - Emergency Department Guidelines
Child Abuse (PIC) - Clinical Practice Manual
Child Protection Unit Protocols - PCH Operational Manual
Child Safeguarding and Protection - CAHS Policy Manual

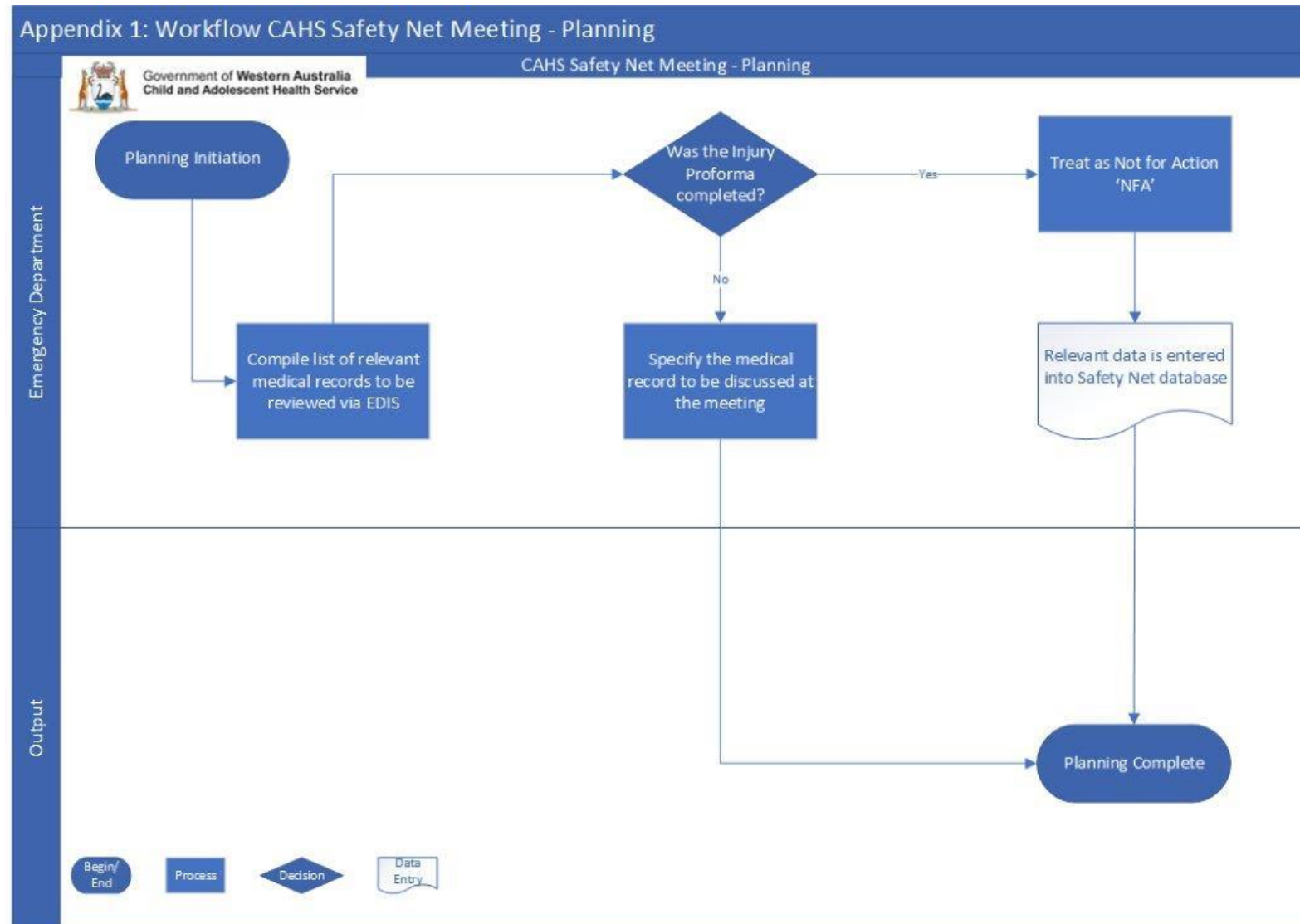
Dental Trauma - Emergency Department Guidelines
Eye Trauma - Emergency Department Guidelines
Fractures - Overview - Emergency Department Guidelines
Head Injury - Emergency Department Guidelines
Immersion - Emergency Department Guidelines
Poisoning Overview - Emergency Department Guidelines
Trauma Serious Injury - Emergency Department Guidelines

Useful resources (including related forms)
Child Safety and Protection Concerns - CAHS Intranet page
Child Protection Unit - PCH Intranet page
Early Childhood Injury Proforma MR301.03
Child protection - Contact Information - Government of WA - Department of Communities – External website
Child Protection Concern Referral Form – Government of WA, Department of Communities Form - External site
Mandatory Reporting of Child Sexual Abuse in WA Western Australian Government - Department of Communities - External website

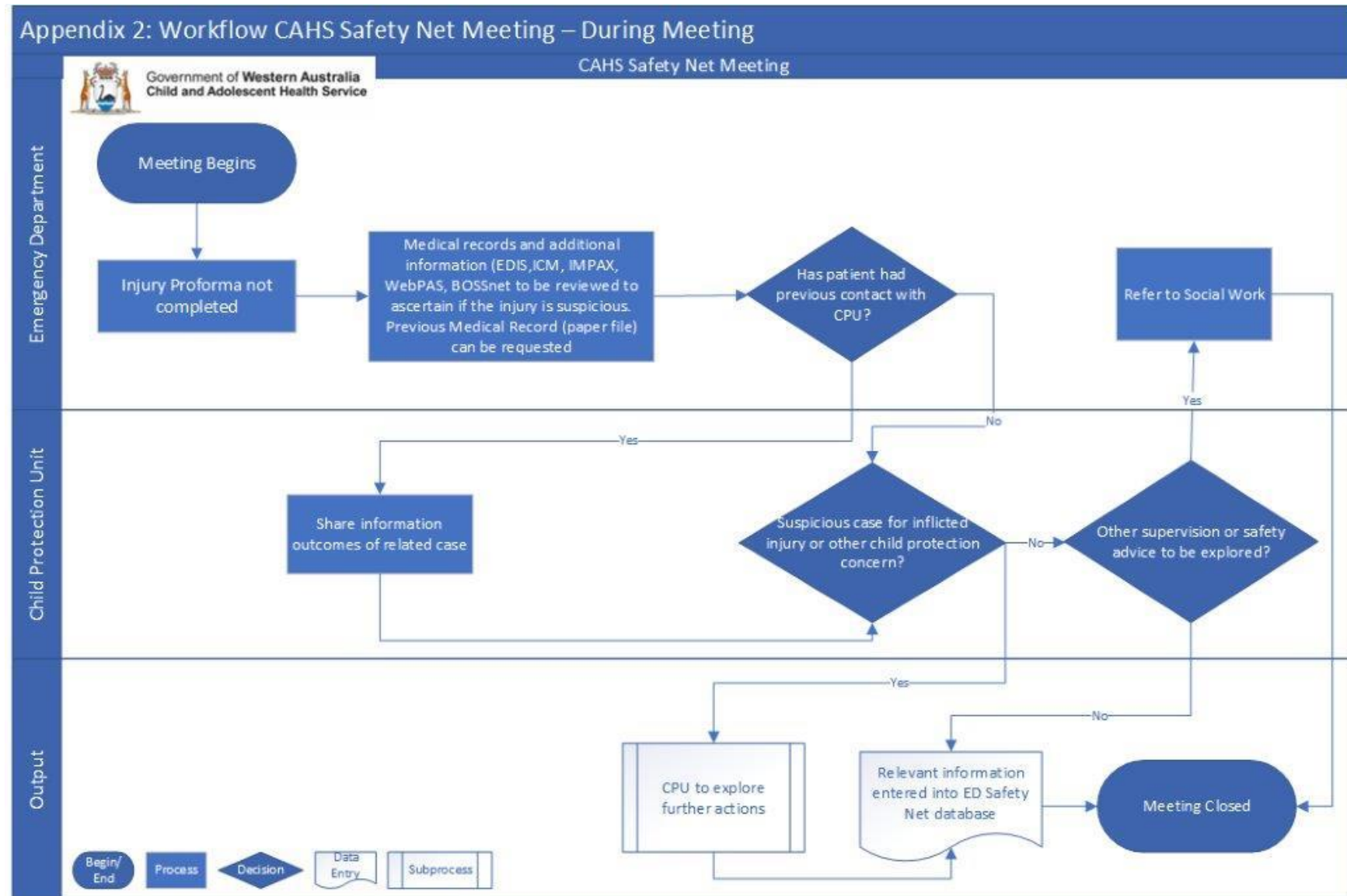
This document can be made available in alternative formats on request.

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Appendix 1: Workflow CAHS Safety Net Meeting – Planning



Appendix 2: Workflow CAHS Safety Net Meeting – During meeting



Appendix 3: Workflow CAHS Safety Net Meeting – Post meeting

